BRIGHTWATER HEALTH PLAN

Prior Authorisation Rules

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1. Purpose and scope

These rules say how Brightwater Health Plan reads a prior authorisation request: how each channel writes an answer, how a request submitted again is counted, how heights, weights, durations, grades and scores are turned into the values a criterion compares, and how each criterion and the screening are worked out.

They apply under every medical policy. The criteria themselves - the ages, weeks, months, grades, scores, indices and conditions a criterion asks for - are set by each service's own medical policy, never by these rules. The worked examples below use figures that belong to no policy.

2. Reading a request

Every request has a request reference, PA- and six digits, and every member a member number, BWH- and seven digits. A request arrives by one of three channels: the provider portal, fax or the EDI feed, recorded as Portal, Fax and Electronic. Its urgency is Standard or Urgent.

A request reference may be written in lower case, as pa-123456, with a space or no hyphen, as PA 123456 or PA123456, or as the six digits alone, and is read as PA- and the six digits. Channels, urgencies and other answers in words arrive in other spellings too, such as Routine for Standard and Expedited for Urgent, and are read as their canonical value.

Each channel writes the member number its own way: the portal as BWH-1234567, sometimes in lower case; the EDI feed without the hyphen, as BWH1234567; a fax as the seven digits alone or as BWH 1234567. Each is read as BWH- and the seven digits. An ICD-10-CM code sent by EDI arrives without its dot, as M5416, and the dot goes back after the third character: M54.16.

A provider may submit a request again before its review begins, to complete an answer it left blank or to correct one. Every submission carries the same request reference and a higher submission sequence. The submission with the highest sequence is the request, wherever it stands among the rows, and every earlier submission is set aside, even when it gives other answers. A row repeated exactly is one row.

An answer written as N/A, NA, n/a, -, unknown or Not recorded is no answer: the item is blank.

3. Height and weight

Height is kept in centimetres and weight in kilograms, each to one decimal place, rounded half up. Each conversion is worked out exactly, and rounded once, at the end.

Height is written 172 cm, 172, 1.72 m or 5 ft 8 in. A foot is 12 inches and an inch is exactly 2.54 cm: 5 ft 8 in is 68 inches, 172.72 cm, kept as 172.7. A height in metres is multiplied by 100: 1.72 m is 172.0 cm.

Weight is written 84.3 kg, 84.3, 186 lb or 13 st 4 lb. One lb is exactly 0.45359237 kg and one st is 14 lb: 186 lb is 84.36818082 kg, kept as 84.4, and 13 st 4 lb is 186 lb, also 84.4 kg.

4. Body mass index

Body mass index is weight in kilograms divided by the square of height in metres. It is worked out from the height and weight kept under section 3, kept to one decimal place, rounded half up, and never taken from a figure the requesting office wrote.

For example, 84.4 kg at 172.7 cm gives 84.4 / (1.727 x 1.727) = 28.2981, kept as 28.3.

A criterion compares the index as kept. At 158.0 cm, 94.9 kg gives 38.0147 and 94.8 kg gives 37.9747; both are kept as 38.0, so both are 38.0 or higher and neither is below 38.0. Compared before it was kept, the second would wrongly be below 38.0.

An index needs both a height and a weight. With either blank, the index is not documented.

5. Treatment durations and months

Conservative treatment is kept in whole weeks. Written in weeks - 9 weeks, 9 wks or a bare 9 - it is that number. Written in days, it is the whole weeks in them: the days divided by 7, the remainder dropped. 154 days is 22 whole weeks and 153 days is 21. A part of a week never counts as a week, and days are never rounded up.

Months since a scan and months in a weight-management programme are whole months: 14, 14 months and 14 mo are all 14.

6. Grades and scores

A Kellgren-Lawrence grade is a whole number from 0 to 4, written 3, Grade 3, KL 3 or KL grade 3.

The Oxford Knee Score runs from 0, the worst, to 48, the best, and is written 22 or 22/48. The Epworth Sleepiness Scale runs from 0 to 24 and is written 15 or 15/24.

A grade or a score beyond its scale - a knee score of 52, a sleepiness score of 27 - is a reading the scale cannot hold. It is treated as not documented, never as a value.

The ASA physical status class is a whole number from 1 to 4, written 2, ASA 2, II or ASA II. Requested inpatient days are whole days, written 3, 3 days or 3 nights.

7. Answers written in words

Yes and No are written many ways, such as Y, yes, True, N, no and False, and are read as Yes and No.

A red flag symptom is written as its name or a common variant: bladder or bowel dysfunction is bowel or bladder dysfunction. Answers such as None, Nil, None reported and No red flags mean no red flag, recorded as No red flag. A blank is no answer, which is not the same as no red flag.

Other conditions are written freely, with abbreviations such as HTN, T2DM, OSA and COPD, other separators and any order. They are read as the plan's twelve condition names, sorted and joined with a semicolon and a space, as in Asthma; Hypothyroidism. The twelve are Asthma, Chronic obstructive pulmonary disease, Depression, Gastro-oesophageal reflux disease, Heart failure, Hyperlipidaemia, Hypertension, Hypothyroidism, Neuromuscular disease, Obstructive sleep apnoea, Osteoarthritis and Type 2 diabetes. Answers such as None, Nil and No conditions mean no other conditions, recorded as No other conditions; a blank is no answer.

A home sleep apnoea test is read as Not done, Inconclusive, Negative or Positive: written Normal it is Negative, Abnormal is Positive, Equivocal is Inconclusive and Not performed is Not done. A psychological assessment is Completed or Not completed, and one that is pending or booked is Not completed.

8. Criterion results

A criterion is Met when the questionnaire shows that it is met, Not met when the questionnaire shows that it is not, and Not documented when an answer it needs is blank.

A criterion that can be met in two ways is Met when either way is met, Not met only when both are known to fail, and Not documented otherwise. Under a policy that asks for 8 whole weeks of treatment unless a red flag is reported: 7 weeks with no red flag is Not met; 7 weeks with the red flag left blank is Not documented; 7 weeks with a red flag reported is Met; and a blank duration with no red flag is Not documented.

A question asked only after an answer - the months since a scan, asked only when there was a scan - is not needed when the first answer settles the criterion. A blank needed answer is Not documented.

The minimum age is read from the member's age on the request.

9. The screening and the decisions open

The screening of a request is REFER TO PHYSICIAN when any criterion is Not met; otherwise REQUEST MORE INFORMATION when any criterion is Not documented; otherwise APPROVE.

A criterion that is Not met decides the screening even when another is Not documented: the request goes to a Physician Reviewer, who reviews the whole request, including what is missing.

The decisions open follow from the screening, as the Utilisation Management Policy sets out: REFER TO PHYSICIAN always; APPROVE only when every criterion is Met; REQUEST MORE INFORMATION only when no criterion is Not met and at least one is Not documented.

For example, criteria found Met, Met and Not documented screen as REQUEST MORE INFORMATION, with REQUEST MORE INFORMATION and REFER TO PHYSICIAN open; Met, Not met and Not documented screen as REFER TO PHYSICIAN, with only REFER TO PHYSICIAN open.

10. Documents and the questionnaire

The questionnaire decides every criterion. The documents sent with a request are evidence: each sentence is read for the kind of evidence it gives, with the evidence guide for that kind.

When a sentence read as conservative treatment gives a number of weeks other than the questionnaire's whole weeks - a letter speaking of eleven weeks of physiotherapy where the questionnaire says 7 weeks - the review flags the disagreement for the reviewer and keeps the questionnaire's answer.

A document never fills a blank answer and never replaces one. When the documents describe an answer the questionnaire left blank, the requesting office is asked for it.

11. The approval likelihood and the expected stay

The approval likelihood is worked out from the member's age, the requesting practice's prior approval rate, the documents sent, how many criteria the screening found Met, Not met and Not documented, the service, the channel, the urgency, the practice's specialty, its network status and the site of service. It is shown to four decimal places and banded on the figure shown, so 0.79996, shown as 0.8000, is HIGH.

The expected stay is worked out for an inpatient operation from the member's age, the ASA physical status class, the body mass index of section 4, whether type 2 diabetes is among the other conditions, how many other conditions there are, the smoking status, the operation, whether the planned hospital runs an enhanced recovery programme and the planned weekday. It is shown to one decimal place. With 3 inpatient days requested, an expected stay shown as 4.1 means a longer stay is expected; one shown as 4.0 does not.

For either figure, an input left blank is filled as the model's training filled it: a number with the median of the training data and an answer in words with its most common value.

12. A worked example

A fax asks for a service under a policy - no Brightwater policy - whose criteria are a minimum age of 21, conservative treatment for 8 whole weeks unless a red flag is reported, and a body mass index below 38.0. The member is 57.

The fax gives the treatment as 52 days: 7 whole weeks. It gives the red flag as Nil: no red flag. It gives the height as 5 ft 10 in, which is 70 inches, 177.8 cm, and the weight as 15 st 7 lb, which is 217 lb, 98.4 kg. The body mass index is 31.1266, kept as 31.1.

The minimum age is Met and the body mass index is Met. The treatment is 7 whole weeks, short of 8, with no red flag, so that criterion is Not met. The screening is REFER TO PHYSICIAN, and REFER TO PHYSICIAN is the only decision open.

Had the fax given 56 days, the treatment would have been 8 whole weeks and every criterion Met. Had it left the red flag blank, the treatment criterion would have been Not documented and the screening REQUEST MORE INFORMATION.